



Exploring the social determinants of health services accessibility during the COVID-19 pandemic in the Huasteca Potosina indigenous communities



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Introduction

The United Nations The-State-of-The-Worlds-Indigenous-Peoples Report remarks that indigenous peoples experience higher ill health, disability, and reduced quality of life, with a much shorter life expectancy than other citizens in the same countries. Their health status severely affects their living conditions, employment and income levels, and access to food, water, and sanitation services. Geographic isolation, poverty, discrimination, and a lack of cultural understanding further contribute to significant structural barriers for indigenous populations to access health care(Who,2023).

Indigenous communities in México have historically faced many healthcare challenges and issues. These include limited access to healthcare facilities, insufficient healthcare infrastructure, cultural and linguistic barriers, health inequalities, lack of health education, and lack of healthcare funding. The situation of indigenous communities in Mexico can vary significantly depending on location, local government policies, and community-specific circumstances; these structural inequalities constituted the main vulnerability factor to the SARS-COV-2 infection, which led to high rates of COVID-19 in Mexican indigenous communities.

The lessons learned during the COVID-19 pandemic emphasize the importance of investigating the social determinants of health that prevent communities from adequate health services. This knowledge will provide information on suitable approaches to strengthen community action and foster interaction and synergies with health authorities and public health decision-makers to address the issues that matter to the community most.

Objective/Research questions

Objective:
Explore the social determinants of health services accessibility during the COVID-19 pandemic in the Huasteca Potosina indigenous communities.

- (Q1) What are the main barriers that prevented members of the Huasteca Potosina indigenous communities from accessing health services during the COVID-19 pandemic?
- (Q2) What health-access issues matter to the communities most, and what are the community-based solutions?



Methodology

We conducted a field study to explore the social determinants of health that prevented Tenek communities of the Huasteca Potosina indigenous communities (Aquismón, Tancanhuitz, San Antonio) from having adequate access to health services during the COVID-19 pandemic. We collected quantitative volunteer data (adapted PSQ-18 questionnaire) regarding general satisfaction, technical quality, health personnel interpersonal manners and communication skills, costs, waiting time, accessibility, and convenience of the local health services. In addition, we conducted ten semi-structured interviews(Marshall GN, Hays RD,1994). The interviews were designed using the phenomenological method and operated using the snowball technique(Martínez,2004) Participants shared their overall perceptions and experiences during the COVID-19 pandemic.

Results

We applied the PSQ-18 questionnaire to 56 volunteers. We also conducted ten semi-structured interviews that averaged 8 minutes in duration.

Lack of timely COVID-19 diagnosis and prompt emergency service

Participants in our study reported needing more access to timely COVID-19 tests and medical treatment to prevent disease spread in their communities. One volunteer mentioned:
"I do not know if I had COVID-19 or not. However, my bones and body, head, waist, back, hands, and fingers hurt. I did not go to the doctor because here there are no, there are no medications, and the doctors do not come often; they come every month; sometimes, they do not even come in the month or even the next month. So, I just bought some paracetamol pills, nothing more."

Lack of timely medical treatment

The participants stated they did not receive timely treatment and denied having access to consultation with a specialist. By their customs and traditions, they carried out treatment through Mexican herbalism. One volunteer mentioned:
"It was tough to have access to access medical attention. People were afraid of people who had COVID. Even doctors and nurses were afraid of the people who contracted the virus. They were afraid of treating us. They did give us decent care. We felt that they discriminated against us. Moreover, they did not have medication they should give people with COVID; they only gave us paracetamol."

Long transfer times to medical units

COVID-19 patients had to travel to hospitals or medical units 1-2 hours away from their homes in their communities. Ono volunteer commented:
"Well, the simple truth is, here it is a struggle to go, you have to pay for the trip to take you there, sometimes there is no money for the fare, and well...well, you just suck it up, you stay here. The closest hospital is more than one hour away."

Question	Median	Mean	Standard deviation
Doctors do not adequately explain what they will do.	3.0	2.95	1.43
The health unit I go to lacks material and equipment.	1.0	1.43	0.97
The medical care I have received is deficient.	3.0	2.57	0.95
Doctors ask me questions that I don't understand.	2.0	2.45	1.20
I pay for the medical care I receive.	1.0	1.68	1.27
The doctors do not adequately explain the treatment of my illness.	3.0	2.93	1.20
I buy the medications that are prescribed to me.	1.0	1.36	0.77
If I need any laboratory study or specialist care, I do not have access to these services.	1.0	1.41	0.87
When I have a medical emergency, I wait a long time to be attended to.	1.0	1.48	1.03
Health personnel do not clearly and precisely speak to me, so I can understand.	3.0	2.77	1.25
Health personnel make me feel discriminated against.	3.0	2.82	1.34
The doctors who treat me do not respect me.	3.5	3.04	1.31
Where I receive medical care, people wait too long.	1.0	1.48	0.87
The place where I receive medical care is far from me.	1.0	1.86	1.17
The health personnel do not listen to me carefully.	3.0	2.91	1.12
Health personnel ignore what I say.	3.0	3.05	1.18
I find it difficult to get an appointment for immediate medical attention.	1.0	1.30	0.83
Health personnel do not speak my native language.	1.0	1.14	0.55
I am not satisfied with the health service in general.	1.0	1.55	1.01
The times I have required medical attention I have struggled to obtain it.	1.0	1.30	0.63
How long does it take to get to a hospital?	3.0 h	3.07 h	0.63 h

Key: 1: Totally agree, 2: Agree, 3: Don't know, 4: Disagree, 5: Completely disagree

Level

Proposed Solutions

Institutional

- Distribute essential medicines through agreements with local institutions and civil associations to access material and equipment, for care in a dignified manner.
- Create monitoring of mobile units to care for people who cannot access medical facilities.

Social

- Promote participation and alliances between community leaders, foster the training of community members as doctors, and nurses, as well as promote prevention campaigns using materials written in indigenous languages.

Community

- Foster a culture of disease prevention through education and led by community health promoters, community leaders, and local community groups.

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Discussion

- Lack of timely COVID-19 diagnosis and prompt emergency service, long transfer times to medical units, and long transfer times to medical units.
- The lack of local medical community services is one of the main reported issues. The closest medical facilities are between one and three hours away from the community and the transportation costs cannot be afforded by many patients. Also, community members recognize that a) the health units that provide care lack materials and equipment; b) they must pay for the pay for medical care they receive, even though they are entitled to receive free medical services; c) they pay for the medications that should receive free medication; d) they do not have access to specialists and laboratory tests; d) emergency services are completely deficient with long waiting times; e) it is hard for them to get an appointment and the medical personnel do not speak their language; f) the average transportation time to medical facilities is about three hours
- The communities find it difficult to develop solutions to these issues without the institutional support established as a mandate by the Mexican constitution. These issues are a structural problem that is very difficult to address without the implementation of concrete institutional programs that address basic community needs like drinking water and suitable roads to communicate with medical facilities. However, palliative measures can be taken at three different levels.